Department of Veterans Affairs
Request for Information
Emergent Virtual Face-to-Face (VF2F) Network

1.0 Purpose

The Department of Veterans Affairs (VA), Veterans Health Administration (VHA) is conducting market research to identify innovative solutions from qualified industry partners for the design, implementation, and/or operation of a comprehensive virtual emergent and urgent care network, and/or its modular components.

2.0 Strategic Vision

VHA envisions a next generation national emergent Virtual “Face-to-Face” (VF2F) care model, potentially serving over 6 million enrollees who seek acute, unscheduled, urgent or emergent care. This program will provide an option to choose on-demand virtual care to meet Veterans where they live. The model will aid staff in determining which Veterans can be served safely and effectively in a virtual setting and which Veterans need acute in-person care. This will expand options for Veterans who are located in rural or highly rural areas or prefer to receive care at home. Additionally, this capability will help reduce the number of low-acuity, in-person emergency care visits, which increase emergency room crowding and wait times for higher-acuity patients and drive increased health care costs[1].

While VHA is open to a comprehensive solution, this RFI takes a modular approach to identify the best options on the market. VHA welcomes submissions from vendors who can meet a subset of requirements. The ideal solution encompasses the entire Veteran virtual emergency care journey. It begins when a Veteran first contacts VHA, through intake, emergent triage, and Veteran registration, into a virtual visit if clinical staff deem it appropriate. It includes capturing patient history and conducting virtual exams, as well as charting, imaging and laboratory order placement, e-prescribing, discharge, and after-visit acute follow up and care coordination as needed. The solution should also support warm hand-offs of Veterans with emergent/urgent health concerns throughout their virtual care journey, facilitate real time collaboration and communication among clinical staff during a patient encounter, and provide care coordination support with the Veteran’s primary care physician and the Patient Aligned Care Team (PACT) in a VHA facility or community care. Solutions should leverage artificial intelligence, software platforms, home-based diagnostics, communication tools, and other innovative features.

The virtual emergency Face-to-Face network will provide Veterans located anywhere in the United States and its territories the ability to connect with an appropriate care clinician on a 24/7/365 basis via a distributed or hub and spoke staffing model. VHA seeks industry input as we are not bound by state licensing rules. Veterans will experience outstanding and convenient on-demand VHA care for their emergent health care needs, helping them get the right care, at the right time, at the right venue.

3.0 Background

VHA is an integrated high reliability health care organization with a network of 170 medical centers and 1300 clinics serving over 9 million enrolled Veterans. VHA providers or VHA remote contract providers can engage in patient encounters across state boundaries throughout the enterprise, regardless of the state in which they hold their license. By leveraging its integrated network and the flexibility of cross-state licensure, VHA can effectively utilize staff at VHA locations or regional or a single national hub to expand access to care for Veterans nationwide through virtual “face-to-face” care services, ensuring timely medical consultations and continuity of care regardless of geographical location.

VHA provided approximately 3.9 million emergency care[2] visits and 1.0 million urgent care visits in fiscal year 2025. This was comprised of 2.2 million ED encounters and 300K urgent care encounters at VHA with the remainder provided by community care. The most common reasons for VHA emergent care encounters are similar to those experienced by the older adult population in the U.S., including chest, abdominal, and musculoskeletal pain; heart failure; infections such as urinary tract or respiratory; and sepsis. Recent studies indicate that 25-38% of ED visits are low acuity, defined as non-emergent medical problems or care related to chronic conditions, and are driven by the patient’s perceived urgency, limited access to alternative care sites, and clinician referrals[3]. A portion of these visits could potentially be safely and effectively resolved through virtual care[4].

4.0 Scope of Solutions

VHA is seeking solutions that address one or more of the following core elements. Each element should be interoperable with existing VHA systems and future vendor systems. Solution element requirements are detailed below in section 5.

4.1 Software Platform for VF2F

Clinical platform should facilitate management of all aspects of the virtual encounter, including triaging, patient tracking, communication and coordination between all care team members involved in the encounter. Staff using the platform should be able to find all information needed and complete all work within the platform, without having to open other tools such as the EHR, Clinical Decision Support Console, etc., as the platform would ideally be interoperable with VHA’s systems. The platform should promote continuity and read/write integration with our existing infrastructure (including both VistA/Computerized Patient Record System (CPRS) and the Oracle Federal Electronic Health Record (EHR), etc. Platform should include tools to improve provider efficiency and the ability to provide more effective, coordinated emergent/urgent care to Veterans. See section 5.1 for the full requirements.

4.2 Staffing Models and Delivery Structure

Models to coordinate and staff a nationally run VF2F (comprised of VHA-employed staff, vendors, or a mix with vendor daily or annual surge capacity). See section 5.2 for the full requirements.

5.0 Requirements for Solutions

5.1 Software Platform for VF2F Requirements

5.1.1 Platform is intuitive for clinicians, employs standard clinical language, facilitates on-demand care delivery, documentation, order entry, care coordination and follow-up, administrative/clinical case review, and operates across the enterprise, regardless of Veteran location in the United States or U.S. Territories. (Note: most Veterans will be accessing virtual emergent/urgent care services from their personal residence using personally-owned equipment [e.g., smart phone]).

5.1.2 Veterans with multiple comorbid conditions, including significant mental health disease burden must be successfully able to navigate the VF2F pathway

5.1.3 Platform enables the delivery of safe, efficient, and effective remote care at the appropriate level to Veterans

5.1.3.1 Platform supports multiple queues, one for each acuity treated by the virtual service (e.g., high, seen by virtual emergency care clinicians; intermediate, seen by virtual urgent care clinicians, or low, schedule for VHA or Community Care Primary Care appointment)

5.1.3.2 Platform supports the ability for an intermediate acuity Veteran to opt for a call-back or scheduled encounter

5.1.3.3 Platform supports care escalation through real-time escalation protocols for critical cases requiring emergent in-person care

5.1.3.3.1 Use of e911 to rapidly engage local emergency services

5.1.3.3.2 Severe mental health concerns

5.1.3.3.3 Sexual assault

5.1.3.4 Platform shall support an anticipated 500k annual virtual care visits with potential future expansion of this system to support 5 million virtual care encounters annually

5.1.4 Platform integration requirements

5.1.4.1 Bidirectional data exchange with the Federal Electronic Health Record (EHR), i.e., CPRS/ Veterans Health Information Systems and Technology Architecture, Oracle/Cerner, and other VHA platforms with negligible lag and full fidelity

5.1.4.2 Joint Legacy Viewer, Virtual Lifetime Electronic Record, and Veterans Data Integration and Federation Enterprise Platform integration may be required

5.1.4.3 Provide a secure, standards-based Application Programming Interface (API) (e.g., Fast Healthcare Interoperability Resources (FHIR) Release 4 (R4) or higher) to support client applications and an ability to integrate with existing VHA APIs. Meeting all requirements may require specific communications beyond FHIR R4.

5.1.4.4 Include smooth integration into clinical medical record workflows, e.g., using Clinical Decision Support hooks or other triggers to make decision support available at the time of decision-making
5.1.4.5 Meets National Council for Prescription Drug Programs standards
5.1.4.6 Health Information Exchange Compliant
5.1.4.7 Platform supports Substitutable Medical Applications, Reusable Technologies (SMART) on FHIR and Health Level 7 integration with Veteran EHRs
5.1.4.8 Register an eligible virtual patient within the EHR to start the encounter
5.1.4.9 Document patient consent to be treated, current location, and call-back number
5.1.4.10 Access chart and patient history in real time
5.1.4.11 Charting /clinical documentation
5.1.4.11.1 Documentation of a virtual medical examination (vital signs if available, patient alertness and symptoms, etc.)
5.1.4.11.2 Automates clinical documentation real time with artificial intelligence (AI)-assisted note preparation, or integrates with existing VHA note preparation tools, such as AI scribe
5.1.4.12 Placing orders (labs, imaging, prescriptions, etc.)
5.1.4.12.1 Orders/studies are routed to the Veteran's closest/preferred in-network location (includes both VHA and community locations) and are communicated to the Veteran's local care team
5.1.4.12.1.1 Ability to route orders to Community Care or VHA locations - including to CBOCs, ACCs, and Medical Centers and ability to communicate these results to the PACT.
5.1.4.12.1.2 Ideally, studies done in the Community Care network (non-VHA care) are routed to this platform in standard C-CDA format and are organized chronologically as it relates to the patient's care journey
5.1.4.12.1.3 Write capability for recording orders in the EHR (Oracle/VistA)
5.1.4.12.2 All laboratory and imaging studies are tracked within the platform to ensure receipt and review of results by virtual emergency and urgent care staff and communication of results to patients
5.1.4.13 Scheduling follow-up visits or consultations with other specialties within VHA or community care through VHA's External Provider Scheduling program
5.1.4.14 Referring low acuity patients to same day (or appropriate short interval) appointments in primary care
5.1.4.15 Plotting longitudinal patient data (patient history for specific labs such as hemoglobin or serum chemistries, etc.)
5.1.5 Platform promotes care coordination and management
5.1.5.1 Built-in care team communication tool to allow virtual emergency/urgent care staff to message within the platform and within the patient encounter to coordinate real time care

5.1.5.2 All Veterans referred to or entering a queue requesting virtual emergency or urgent care services are tracked from the moment of referral or request until the patient is fully discharged from the encounter. Patient orders are tracked and when results are available, the on-duty care team is alerted. There is a method for critical results to be flagged for expedited follow up by the on-duty virtual care team.

5.1.5.3 Displays (in real-time) the queue of patients awaiting and receiving virtual emergency/urgent care services, similar to in-person emergency department tracker boards, maintaining sensitivity to time given that virtual care does not have a defined space. Therefore, it ensures that patients are easily visible and tracked, and not “lost” within the tracker to the care team.

5.1.5.3.1 Includes the Veteran’s chief complaint, status of care, wait time, care team, and care coordination items such as comments or orders placed.

5.1.5.3.2 If routed through the VHA Health Connect/Clinical Call Center, the platform must display that a patient was referred

5.1.5.4 Ensures minimal wait times with queue management and the ability to re-prioritize patients based on need

5.1.5.5 Promotes seamless handoff of Veteran between care team members during encounter

5.1.5.6 Supports multiple users interacting with the display system with no data collisions/write-failures, including multiple providers working within an individual patient’s record (i.e. display system must be able to pass the Atomicity, Consistency, Isolation, and Durability test)

5.1.5.7 Integrates with MyHealthEVet to send secure messages and after-visit summaries to Veterans, or supports a method to send on-demand secure documents and messages containing PHI to patients within hours of the index encounter

5.1.5.8 Platform facilitates prescribing of medication

5.1.5.8.1 Platform includes options for filling prescriptions through VHA pharmacies or the central mail-order pharmacy

5.1.5.8.2 Platform includes an e-prescribing solution to facilitate sending prescriptions to external pharmacies

5.1.5.8.3 Platform ensures that prescription generation is aligned with the VHA formulary and outpatient pharmacy network

5.1.6 Platform has a validated Decision Support Tool (DST), or integrates with another DST algorithm such as ClearTriage

5.1.6.1 Veterans contacting the service directly must be triaged by a member of the care team

5.1.6.1.1 If AI is proven safe and effective, then platform leverages intelligent triage and routing assistance, to include AI-driven triage assessment or large-scale data analytics engines to recommend patient acuity based on triage conversation between the Veteran and clinical staff

5.1.7 Platform supports multi-modal communication between Veterans and providers

- 5.1.7.1 Platform has native video, phone, chat, and text messaging capabilities and integrates with other existing VHA tools such as Genesys, Cisco Jabber, and VA Video Connect
- 5.1.7.2 Veterans can access visits via mobile app, web portal, telephone, or smartphone with minimal connection interruptions and the ability to seamlessly reconnect without multiple validation/verification steps
- 5.1.7.3 Platform is oriented as video-first with alternate access pathways for Veterans without video capabilities. Video capability meets or exceeds industry standards for successful video encounters.

5.1.8 Platform captures data for key performance indicators and integrates it into the VHA Corporate Data Warehouse to support patient safety and continuous improvement

- 5.1.8.1 Measures process and outcome-based parameters accurately for individual encounter to enterprise-level reporting needs
- 5.1.8.2 Quality assurance and clinical governance oversight; including display of clinical quality metrics and health outcome stacking
- 5.1.8.3 System performance monitoring and real-time failure signal recognition and escalation
- 5.1.8.4 Financial performance and utilization analytics

5.1.9 Patient safety event management: interfaces with existing VHA systems (such as Joint Patient Safety Reporting System under the National Center for Patient Safety)

5.1.10 Platform generates real-time operational dashboards (patient volume, wait time, open orders, etc.)

5.1.11 Supports customization and configuration to better support Veteran care needs and VHA business practices

5.1.12 Platform adheres to the principles of clinical software design such as those established by the Office of National Coordinator for Health Information Technology (ONC)

5.1.13 Vendor utilizes application performance monitoring strategies for application health and performance, and develops product performance metrics that will be monitored throughout use of the application

5.1.14 Vendor develops a system to allow users to provide feedback, request support, and suggest improvements

- 5.1.14.1 Support metrics include:
  - 5.1.14.1.1 Number of issues reported
  - 5.1.14.1.2 Problem type identification
  - 5.1.14.1.3 Issues resolved (%)
  - 5.1.14.1.4 Time to resolution
  - 5.1.14.1.5 Priority
  - 5.1.14.1.6 Severity
  - 5.1.14.1.7 Performance against metrics
  - 5.1.14.1.8 Suggestions to reduce help requests
  - 5.1.14.1.9 Summarized feedback
- 5.1.14.2 70% of help requests shall be resolved within 15 minutes
- 5.1.14.3 All help requests shall be resolved within two business days
- 5.1.14.4 Software is fully available and not in “downtime” 99.9% of the time, 24/7/365

5.1.15 Knowledge portal that collates up-to-date information about VHA and community resources adjacent to the Veteran's physical location, anywhere within the U.S. and territories where Veterans are located

5.1.15.1 Types of facilities within knowledge base include:

5.1.15.1.1 Urgent, emergent, primary, and specialty care centers

5.1.15.1.2 Medical centers, hospitals, and outpatient clinics

5.1.15.1.3 Pharmacies

5.1.15.1.4 Diagnostic laboratories

5.1.15.1.5 Imaging centers

5.1.15.2 Portal should provide the following details about each resource:

5.1.15.2.1 Operating hours and status at time of call (open/closed, wait time if available, etc., including holiday hours)

5.1.15.2.2 Address and distance / current drive time from Veteran

5.1.15.2.3 Accurate list of services provided (Computed Tomography, Magnetic Resonance Imaging, orthopedics, dermatology, outpatient pharmacy, etc.)

5.1.15.2.4 Types of insurance accepted, including in or out of network for VHA health care

5.1.15.2.5 Preferred contact method (phone, email, etc) and name of lead personnel, if applicable (e.g., chief of service line, provider in triage)

5.1.15.3 Tool must be accurate and reflect changes to clinic status in real-time

5.1.16 Offers AI virtual assistant options, leveraging AI or Agentic AI to help collate relevant information from the chart, gather resources (such as VHA Directives), VHA Medical Center or community facility information (such as detailed in 5.1.14

5.1.17 Supports multi-language support and accessibility features

5.1.18 Integration/interfacing with vSignals data to display patient satisfaction metrics to identify opportunities for improvement

5.1.19 Predictive modeling for capacity planning

5.1.20 Identify Veterans who frequently use emergency departments and implement proactive strategies to manage their care more effectively

5.2 Staffing Models and Delivery Structure Requirements

5.2.1 Veterans must be able to access acute, unscheduled, urgent or emergent care with maximal convenience, with or without nurse-led triage, through app-, phone-, and web-based portals of entry anywhere in the United States and its territories on a 24/7/365 basis

5.2.1.1 Can be accomplished via VHA staff, contract staff, or a mixture of both

5.2.1.2 Vendor must allow for treatment of Veterans in any state or territory, navigating issues of contracted provider state licensure. As federal employees, VHA Staff can legally provide Veteran care across state lines

5.2.1.3 Vendors are encouraged to present solutions that realize efficiencies from regional (hub-and-spoke) or national (distributed) coverage strategies, given the capability of VHA providers to practice across state lines

5.2.1.4 Solution provides Veteran-facing and clinical staff-facing training modules to optimize understanding and use of the program

5.2.2 Veterans must rate the VF2F care service experience at an outstanding level as determined by a vSignals Veteran experience overall satisfaction score of 85% or greater nationally

5.2.3 Veterans can contact virtual emergency/urgent care service directly via telephone, phone app, chat, or web browser interface. They can also be referred to the virtual emergency/urgent care service via a warm handoff from a nurse triage line (e.g., VHA Clinical Call Center / VA Health Connect)

5.2.4 All handoffs of the Veteran between care team members must be seamless, including warm hand-offs for high acuity patients wherein contact is not lost with the Veteran during the encounter

5.2.4.1 To 988 / Veteran's crisis line

5.2.5 Wait times from time of first VHA contact to time of transfer to a Licensed Independent Provider (LIP) should fall within set VHA guidelines

5.2.5.1 Veterans accessing the VF2F service directly (i.e. not via VA Health Connect nurse triage) should experience no more than a 5-minute median wait time to receive initial contact with an emergency care clinician

5.2.5.2 Once triaged, <10 minutes to LIP for high acuity, and <15 minutes for intermediate acuity

5.2.5.3 Contingency plans must be in place for surge demand that places wait times outside of the acceptable limits

5.2.6 Veterans must be able to access appropriately certified care providers for the level of care needed

5.2.6.1 Appropriate mix of emergency medicine, urgent care physicians/providers and nurses, as well as pharmacy and care coordination staff

5.2.6.1.1 Staff must be board-certified, licensed, credentialed, and privileged

5.2.6.1.2 Physicians and advanced practice providers must maintain Emergency Medicine competency and licensure, along with having at least one in-person shift in an emergency department per month

5.2.7 Maintain current VHA levels of care

5.2.7.1 Successfully resolve a Veteran's acute care concern (without needing in-person emergency department/urgent care [ED/UC] care escalation) in over 50% of cases through a completed virtual EC/UC encounter

5.2.7.2 At least 20% of Veterans triaged as needing an emergent or urgent level of care after nurse triage are offered a virtual on-demand emergency or same day urgent care encounter

5.2.7.3 For Veterans who have their acute care concern resolved through a completed virtual emergency/urgent care encounter, no more than

10% experience a subsequent ED visit within 7 days of the virtual EC/UC encounter

5.2.7.4 Expected handle times are 1.5 – 3 Veteran encounters per clinician hour

5.2.8 Personnel Requirements:

5.2.8.1 Provide safe, efficient, and effective care at the appropriate level. Includes on-demand virtual emergency and urgent care, or directing Veterans to an in-person (local VHA or community) urgent care or emergency department, as clinically appropriate

5.2.8.2 Clinicians must be capable of evaluating and treating or referring Veterans with high acuity concerns (including chest pain, shortness of breath, acute ocular symptoms, etc.) as might be seen in an emergency department setting

5.2.8.2.1 Includes utilizing e911 capabilities to rapidly engage local emergency services)

5.2.8.3 Staff must be familiar with management of Veteran acute mental health concerns and VHA resources, policies, and procedures to ensure appropriate care escalation and/or clinical care coordination for such concerns

5.2.8.4 Cultural competency and Veteran patient population experience preferred

5.2.8.5 Staff must follow VHA policies and directives

6.0 References

1. Schull MJ, Kiss A, Szalai JP. The effect of low-complexity patients on emergency department waiting times. Ann Emerg Med. 2007;49(3):257-264, 264.e1. doi:10.1016/j.annemergmed.2006.06.027

2. Vashi, A. A., Bozkurt, S., Urech, T., Wu, S., Asch, S. M., & Tran, L. D. (2025). Persistent Emergency Department Use Among Veterans: Longitudinal Patterns and Opportunities for Targeted Intervention. Military medicine, usaf479. Advance online publication. https://doi.org/10.1093/milmed/usaf479

3. Ramachandran A, Tran LD, Asch S, et al. Emergency Department Utilization by Veterans for Low-Acuity Conditions After Virtual Care Expansion. JAMA Netw Open. 2025;8(11):e2545696. https://doi:10.1001/jamanetworkopen.2025.45696

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